BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Safety and response tests have different purposes. Use the individual prescription and appointment dates.

Check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Blood and organ safety	Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal.	Confirm baseline results and the starting plan.	Regular blood count, kidney and liver tests, blood glucose and bone blood profile. National Cancer Control Programme (NCCP) does not specify one interval for these tests. Review symptoms, PSA and testosterone, with scans according to the individual plan.	Confirm results and symptoms before the next prescription or injection.	Review bone, metabolic and heart health.
Treatment and appointments	Agree treatment aim, formulation, duration and medicines list.	A trained healthcare professional places the implant under the skin of the front of the abdomen. This 10.8 mg implant has a different schedule from the 3.6 mg formulation; do not swap strengths or appointment intervals.	Keep an accurate medicine / injection record.	Tell the team if you take blood-thinning medicines or are very thin. These can increase bleeding risk at implantation. Severe abdominal pain, marked swelling, dizziness or collapse afterwards needs urgent help. Do not stop blood thinners yourself.	Continue until the team changes the plan.
Cancer response	Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.	Document the baseline PSA and clinical picture.	Ask when the next PSA and any testosterone test are due.	Review symptoms and results together.	Scans follow the individual plan; there is no universal interval.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00477 v3: official regimen · Zoladex LA: product information

Tests in your plan

Blood count, kidney and liver tests

WHEN

Regular blood count, kidney and liver tests, blood glucose and bone blood profile. NCCP does not specify one interval for these tests. Review symptoms, PSA and testosterone, with scans according to the individual plan.

Checks blood cells and organ health.

Blood glucose and bone blood profile

WHEN

Regular blood count, kidney and liver tests, blood glucose and bone blood profile. NCCP does not specify one interval for these tests. Review symptoms, PSA and testosterone, with scans according to the individual plan.

Checks metabolic effects and blood minerals.

PSA and testosterone

If part of your plan

WHEN

Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.

Checks cancer activity and hormone suppression.

electrocardiogram (ECG) and bone-density assessment

If part of your plan

WHEN

ECG (heart tracing) and bone-density scans are arranged according to heart-rhythm risk, fracture risk and clinical need. Your team confirms which tests you need.

Checks heart rhythm or fracture risk when indicated.

THE WHOLE TREATMENT JOURNEY

Goserelin 10.8 mg: hormone implant every 12 weeks: treatment and checks

Locally advanced or metastatic hormone-sensitive prostate cancer under NCCP 00477.

Goserelin 10.8 mg is placed under the skin of the front of the abdomen every 12 weeks (84 days). Your team writes the next date and the intended treatment duration.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Blood and organ safety NCCP · SMPC	Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal.	Confirm baseline results and the starting plan.	Regular blood count, kidney and liver tests, blood glucose and bone blood profile. NCCP does not specify one interval for these tests. Review symptoms, PSA and testosterone, with scans according to the individual plan.	Confirm results and symptoms before the next prescription or injection.	Review bone, metabolic and heart health.
Treatment and appointments NCCP · SMPC	Agree treatment aim, formulation, duration and medicines list.	A trained healthcare professional places the implant under the skin of the front of the abdomen. This 10.8 mg implant has a different schedule from the 3.6 mg formulation; do not swap strengths or appointment intervals.	Keep an accurate medicine / injection record.	Tell the team if you take blood-thinning medicines or are very thin. These can increase bleeding risk at implantation. Severe abdominal pain, marked swelling, dizziness or collapse afterwards needs urgent help. Do not stop blood thinners yourself.	Continue until the team changes the plan.
Cancer response NCCP · SMPC	Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.	Document the baseline PSA and clinical picture.	Ask when the next PSA and any testosterone test are due.	Review symptoms and results together.	Scans follow the individual plan; there is no universal interval.

What each check is for

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Treatment and review

One implant every 12 weeks

This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

Ask / act: Regular blood count, kidney and liver tests, blood glucose and bone blood profile. NCCP does not specify one interval for these tests. Review symptoms, PSA and testosterone, with scans according to the individual plan.

Sources: NCCP · SMPC

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-14 · Source reconciled draft; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.52